



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

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HIGHLIGHTS (last 24 hours)



At the end of the day on September 15, 2026, 59 new confirmed cases of Ebola Virus Disease (EVD), including 16 community deaths, were reported in the provinces of Ituri (35 cases), North Kivu (21 cases), and Bas-Uélé (2 cases) and Tshopo (1 case). In addition, 16 deaths within the CTE were recorded.

In the last 24 hours, no new health zones have been affected.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
7,404	3,577	48.3%	930	1,776	87.4%



Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



62 out of 167
health zones affected
(37.1%)

Summary of key points from the last 24 hours

A total of **59 new confirmed cases** of Ebola Virus Disease (EVD), including **16 community deaths**, were reported in the last 24 hours. These confirmed cases are distributed across the provinces of **Ituri (35 cases)**, **North Kivu (21 cases)**, **Bas-Uélé (2 cases)**, and **Tshopo (1 case)**. Conversely, **16 deaths in Ebola Treatment Centers (ETCs)** were reported, including 10 in Ituri and 6 in North Kivu, bringing the total number of deaths for the day to **32**.

Twenty-three (23) patients were declared cured after obtaining two negative results for Ebola virus disease, respectively **13 in Ituri**, **6 in Haut-Uélé** and **4 in Nord-Kivu**.

From the start of the epidemic until September 15, 2026, **there were 7,404 confirmed cases, including 3,577 deaths**, representing a case fatality rate of **48.3 %**. The epicenter remains the province of Ituri, which accounts for **77.7%** of the confirmed cases reported in the seven affected provinces and also records **59.3%** of new confirmed cases.

No new health zones have been affected in the last 24 hours.

The DRC continues to have **62 out of 167 health zones (37.1%)** affected and **7 provinces impacted**. Ituri accounts for 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 15, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	35	5751	2,622	45.6%	28/36 (77.8%)
North Kivu	21	1323	821	62.1%	16/34 (47.0%)
Haut-Uélé	0	290	118	40.7%	6/13 (46.1%)
Tshopo	1	29	10	34.5%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	2	7	4	57.1%	3/11 (27.3%)
South Ubangi	0	1	1	100.0%	1/16 (6.3%)
Total	59	7,404	3,577	48.3%	62/167 (37.1%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 15, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5751	2622	45.6%	35	8	10	18
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	179	39	21.8%				0
Boga	2	2	100.0%				0
Bunia	1611	510	31.7%	8	2		2
Damascus	40	9	22.5%	1	0		0
Drodro	15	7	46.7%				0
Fataki	79	30	38.0%	1	0		0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	136	88	64.7%				0
Lita	264	130	49.2%				0
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	57	32	56.1%	1	1		1
Mangala	322	146	45.3%	7	0		0
Mongbwalu	670	307	45.8%	4	2		2
Nia-Nia	248	128	51.6%	3	2		2
Nizi	742	273	36.8%	5	0		0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0

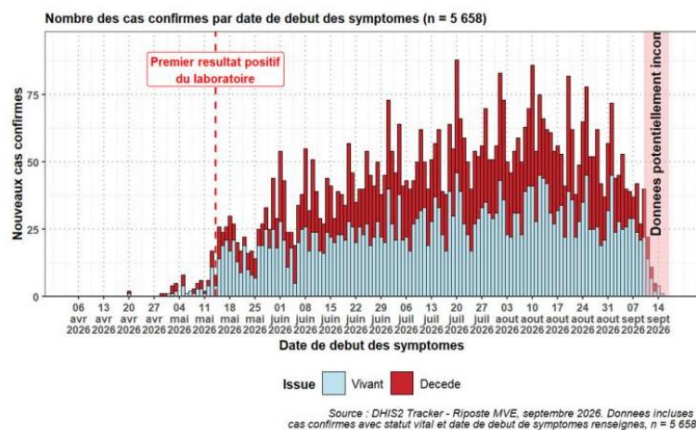
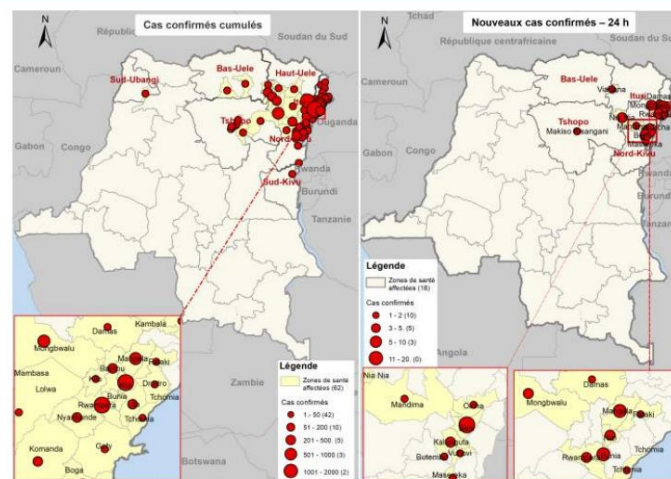
Rwampara	1029	350	34.0%	3	1		1
Tchomia	75	25	33.3%	2	0		0
To ventilate	N / A	456	N / A			10	10
North Kivu	1323	821	62.1%	21	7	6	13
Blessed	241	178	73.9%	10	6	2	8
Good	18	7	38.9%				0
Butembo	242	157	64.9%	2	0		0
Goma	1	0	0.0%				0
Kalunguta	36	12	33.3%	4	0		0
Katwa	533	337	63.2%				0
Kayna	1	0	0.0%				
Kyondo	30	20	66.7%	0	0	1	1
Lubero	15	7	46.7%				0
Mabalako	19	9	47.4%				0
Manguredjipa	9	3	33.3%				0
Masereka	23	9	39.1%	2	0		0
Musienene	117	64	54.7%				0
Mutwanga	6	2	33.3%				0
Oicha	12	10	83.3%	1	1	2	3
Vuhovi	20	6	30.0%	2	0	1	1
Haut-Uélé	290	118	40.7%	0	0	0	0
Boma Mangbetu	34	15	44.1%				0
Gombari	3	1	33.3%				0
Isiro	98	42	42.9%				0
Pawa	61	26	42.6%				0
Rungu	2	1	50.0%				0
Wamba	92	33	35.9%				0
Tshopo	29	10	34.5%	1	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	11	5	45.5%	1	0	0	0
Mangobo	4	2	50.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	7	4	57.1%	2	1	0	1
Buta	1	1	100.00%				0
Viadana	3	1	33.33%	2	1	0	1
Ganga	3	2	66.67%				0
South Ubangi	1	1	100.0%	0	0	0	0
Bulu	1	1	100.0%				0
Total	7404	3577	48.3%	59	16	16	32

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for September 15, 2026

Table 3: Status of alerts notified by provinces for September 20, 2019									
DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	21	5	26	6	5	15	0	11	5
Bas-Uélé	36	1	37	14	0	22	0	14	8
Ituri	1105	71	1176	133	69	486	1	202	95
North Kivu	1,125	56	1181	137	50	849	6	187	76
South Kivu	12	0	12	0	0	12	0	0	0
South Ubangi	3	0	3	3	0	0	0	3	0
Tshopo	155	0	155	6	0	149	0	6	1
Total General	2,457	133	2,590,299		124 1 533		7	423	185

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

Over the past 24 hours, **2,590 alerts** were recorded across the seven affected provinces. Of these, 1,963 (75.8%) were verified. All **423 (21.5%) alerts were validated as suspected cases**, and all of these were investigated. Of these, 185 were referred to the CT/CTE (Table 3).

The proportion of contacts followed is at 87.4% (28,065/32,094) compared to the previous day (89.3%); 91.4% in Ituri (16,066/17,576), 83.3% in North Kivu (11,146/13,386), 81.1% in Haut-Uélé (446/550); 75.5% in Tshopo (222/294); 67.9% in Bas-Uélé (169/249) and 41.0% in Sud Ubangi (16/39).

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

Two (2) alerts were notified to the PoE/PoC in the last 24 hours, one (1) in Ituri and one (1) in North Kivu. **In Ituri**, a live alert was intercepted at the Tchaï PoC (Makabo Health Zone) en route to Bunia, presenting with muscle pain, severe fatigue, and a cough; the alert was notified, validated, and the patient transferred to the CME's Emergency Medical Center (EMC). **In North Kivu**, an alert notified to the Kasitu PoC (Beni Health Zone) was invalidated after further investigation; the Butuhe PoC (Kalunguta Health Zone) was activated, and construction work on shelters continues at the Kasitu and Mukulia PoCs.

Table 4. Status of key PoE/PoC indicators by province, as of September 15, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	96.6% (58/60) 100%	(20/20) 55.6% (5/9)	60.0% (18/30) 63.6% (7/11) 83.1% (108/130)			
% of functional strategic PoE/PoC	100%	100%	100%	100%	54.0%	90.8%
Number of people who have passed PoE/PoC	146,808	92,480	7,555	15,978	12,699	275,520
% of travelers screened	98.0%	98.2%	98.5%	98.5%	97.0%	98.1%
% of travelers who washed their hands	96.9%	98.2%	98.5%	98.4%	99.0%	97.6%
% of travelers made aware	99.5%	98.2%	100.0%	100.0%	100.0%	99.1%
Number of alerts notified	1	1	0	0	0	2
Number of validated live alerts (suspected cases)	1	0	0	0	0	1
Number of bodies intercepted today % of suspected	0	0	0	0	0	0
cases transferred to CT/CTE	100% (1/1)	N / A	N / A	N / A	N / A	100% (1/1)
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A	N / A	N / A
% of suspected cases investigated within 24 hours	100% (1/1)	100%	N / A	N / A	N / A	100% (1/1)

1.2.2. Challenges

Suboptimal operationalization of PoE/PoC coupled with low reporting completeness in Tshopo (60.0%, 18/30), South Kivu (55.6%, 5/9), and Haut-Uélé (63.6%, 7/11); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri) and the ongoing strike due to non-payment of service providers for the Nzebi PoC; low alert encoding in DHIS2 Tracker in Ituri. In Tshopo, 18 PoCs were activated out of the 30 planned; in Haut-Uélé, 4 PoE/PoCs (Dusu, Apodo, Aungba, and Gossamu) did not report due to lack of connectivity. In South Kivu, isolation devices were not in place at the Kavimvira, Kalundu Port SNCC, and Baraka PoEs. In Bas-Uélé, 14 PoCs activated and PoEs reinforced out of 41 (34.1%), lack of trained health personnel and problem of mobility and connectivity to PoE/PoCs.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 35 new positive results** (27 living and 8 deaths) out of 177 new samples received and analyzed (**positivity: 19.8%**).
- **North Kivu: 21 samples received and tested** (14 live and 7 dead) out of 160 samples analyzed (**positivity of 13.1%**).

- **Haut-Uélé: 13 samples received and analyzed, all revealed to be negative.**
- **Tshopo: 1 new positive result** (1 live sample), out of 6 samples received and tested (positivity: 16.7%).
- **Bas-Uélé: 2 new positive results** (1 living and 1 death) out of 6 samples received and analyzed at the Isiro laboratory (positivity: 33.3%).
- **South Ubangi:** Three samples were taken from the suspects. The results are pending.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 26 of the 42 planned septic tanks were opened (61.9%), 89 of the 93 identified households were decontaminated (95.7%), and all 8 health facilities were decontaminated (100%); 33 hygiene kits were distributed in Mongbwalu. Regarding the health surveillance system (HSS), 83 alerts were recorded, 66 HSS tests were carried out out of 75 (88.0%), and 61 of 66 bodies were swabbed (92.4%).
- **In North Kivu**, 15 of the 27 planned screenings were opened (55.6%), 10 of the 21 identified households were decontaminated (47.6%), and 18 of the 25 health facilities were decontaminated (72.0%); 6 scorecard assessments were conducted with an average score of 46.5% (3 in Musienene, 1 in Kalunguta, 1 in Butembo, and 1 in Vuhovi); 45 handwashing stations were installed in 2 schools, 1 mosque, and 1 church in Katwa with UNICEF support; 594 service providers were briefed, 184 community health workers were monitored and supported, 396 handwashing stations were monitored, 11 triage units were installed, and 560 liters of water were supplied in Oicha. Regarding blood and body discharge (BBD), 58 alerts were recorded, 61 BBDs were conducted, and 58 bodies were swabbed (100%).
- **In Tshopo**, 3 households in Bafwandambo were identified for decontamination, and 10 household kits were distributed to this health area; 59 community health workers (CHWs) in the Makiso-Kisangani health zone were briefed on infection prevention and control (IPC); and 13 community health workers (CHWs) were monitored and supported in 4 health zones (Makiso-Kisangani, Mangobo, Tshopo, and Kabondo), including briefings for 153 service providers and monitoring of 12 handwashing stations. Regarding the health district (HDR), one alert was recorded for a deceased person at the Horizon Medical Center (Makiso-Kisangani), but a body could not be swabbed due to the lack of mobilization of the sampling team.
- **In Haut-Uélé**, 6 out of 9 planned testing sites were opened (66.7%), 26 out of 32 identified households were decontaminated (81.3%), and all 6 health facilities were decontaminated (100%), in the health zones of Isiro, Pawa, Boma Mangbetu, and Wamba; 5 facilities were assessed for quality score, and one facility received an IPC (Infection Prevention and Control) kit. Regarding the SDS (Single Diagnosis System), 6 alerts were recorded, 5 SDSs were carried out, and 5 bodies were swabbed; one SDS could not be completed due to resistance.
- **In Bas-Uélé**, 2 rings were opened out of 2 expected (100%) in Viadana, but neither of the 2 households identified nor the identified FOSA was decontaminated; briefing of 23 PPLs and 3 hygienists and follow-up of healthcare providers from 6 ESSs in the ZS of Ango, Monga and Ganga.

1.4.2. Challenges

- **In Ituri** : low opening of rings (61.9%; 26/42); insufficient decontamination teams in the health zones of Komanda and Mangala; community resistance in Rwampara; lack of PPE in Lita; 10 bodies not swabbed in Fataki and Mandima due to lack of inputs and teams as well as refusal by families.
- **In North Kivu** : decontamination not covered in the health zones of Biena, Masereka, Kyondo, Mabalako and Vuhovi due to a lack of activation, training, and motivation of the teams; lack of mobility for the deployment of provincial IPC teams; urgent need for training

for all PCI Data on data collection and management tools; non-alignment of 86.6% of PCI WASH pillar members on the INSP motivation list; insufficient handwashing facilities in schools.

- **In Tshopo** : insufficient PCI inputs in households, schools, markets and churches; low proportion of SSE with an PCI score above 80% (2.2%; 2/92) and incinerator at HGR Makiso-Kisangani still out of order.
- **In Haut-Uélé** : lack of community PCI kits, lack of triage units in ESS and poor timeliness of daily data from health zones.
- **In Bas-Uélé** : absence of compliant waste management areas and sorting units in the 20 ESS of 5 health hotspot zones; insufficient bins and handwashing facilities, untrained IPC providers in the 5 priority health zones (Buta, Ganga, Viadana, Titule and Poko) and absence of IPC supervisors.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, 82 new admissions were recorded in the various dedicated facilities, 103 discharges (13 recoveries). Thus, 461 patients are being treated in hospital for 1015 beds (occupancy rate of 45.4%); 432/451 (95.7%) patients received hot meals; 9/9 (100%) children are under LNPE/SLM at the CTE, 3/3 (100%) at the nursery and 36/36 (100%) in the community.
- **In North Kivu**, 118 new admissions were recorded compared to 93 discharges, including 4 recoveries; 373 patients are being treated in hospital, including 216 in dedicated facilities, representing an overall occupancy rate (94.7%) close to saturation; 30/30 (100%) patients (16 confirmed and 14 suspected) received a hot meal, as well as 15/15 (100%) accompanying persons at the CTE in Beni (7) and the CT in Oicha (8) and 52/52 (100%) frontline providers; 9/9 (100%) non-breastfed orphans are on breast milk substitute (CTE Beni: 4, CT Kalunguta: 4 and CT Oicha: 1).
- **In Haut-Uélé**, 6 new admissions were recorded against 17 discharges (6 recovered). Thus, 63 patients are being treated in hospital for 136 beds, representing an occupancy rate of 46.3%; 73 accompanying persons and 31 PPLs benefited from three hot meals, except in the Rungu and Gombari health zones.
- **In Bas-Uélé**, five (5) new admissions were recorded against one discharge (1 death); 11 patients remain in isolation, including 2 confirmed cases taken care of at the HGR of Ganga; 9 high risk contacts are under prophylaxis in Buta and Ganga.
- **In Tshopo**, no new admissions and no discharges were recorded. Six (6) Patients are in isolation for 31 beds, representing an occupancy rate of 19.4%; 6 suspected cases received a hot meal, 1 confirmed unstable patient received a specific diet and 42/42 (100%) frontline providers were served.
- **In South Kivu**, there were no new admissions. Sixteen (16) patients are in isolation out of 25 beds. resulting in an occupancy rate of 64.0%.

1.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 42.1% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 2,544 people have been vaccinated to date out of 11,703 targeted TPL/PPL (21.7%), including 1,495 in Makiso-Kisangani, 368 in Mangobo, 331 in Kabondo, 170 in Tshopo and 96 in Lubunga.
- **In Bas-Uélé**, of the 500 doses of ERVEBO vaccine received at the PEV antenna in Buta, 50 remained in Buta and 450 were deployed to the ZS of Ganga for the vaccination of PPL; a team composed of the MCA, the CO SGI and the national supervisor was deployed to Ganga in support.
- **South Ubangi**, Acquisition of 300 doses of ERVEBO vaccine at the PEV branch; briefing of central and provincial supervisors on Ebola vaccination among PPL; preparations for the start of PPL vaccination.
- **In Ituri**, preparations for the vaccination of PPL continue.

1.6.2. Challenges

Low uptake of vaccination among the local population and insufficient management tools **in Tshopo**, where the Bafwasende vaccination site remains inactive. **In Bas-Uélé**, there is no micro-plan for vaccinating the local population in the six priority health zones (Aketi, Ango, Bili, Buta, Ganga, and Viadana).

1.7. Continuity of Care

- **In Tshopo**, 137 cases received the experimental treatment, including 18 confirmed cases and 119 receiving post-exposure prophylaxis (81 treatments completed, 11 interrupted, and 27 ongoing); development of the recovery plan for the Kisangani Free University Hospital Center (CHU-LIKIS), finalization of the plan for the Lindi Health Center (Bafwasende), and receipt of medications from the UG-PDSS project for the Tshopo Health Zone. A fifth confirmed case of PPL from the Boyoma Health Center was recorded, and the majority of health centers in Bafwasende remain without personal protective equipment (PPE).
- **In Haut-Uélé**, continued briefing of service providers on the specific and prophylactic treatment of Ebola virus disease and preparation of lists of recovered patients awaiting discharge as well as lists of PPLs according to their status.
- **In Bas-Uélé**, follow-up of contacts who have adhered to compassionate prophylaxis in Buta and Dingila and post-exposure prophylaxis of PPL high-risk contacts in the Ganga health zone.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 4,894 home visits reached 19,702 people and 1,135 educational talks reached 15,822 people; 71 mass awareness campaigns reached 4,292 people, 42 community debates reached 592 people and 10 advocacy campaigns reached 92 people; 239 community alerts were notified (including 45 deaths); 27 of the 35 feedback issues were resolved within 48 hours and 20 of the 37 community incidents were resolved (completeness: 12/28 health zones; 43%); support was provided for the pre-listing of 517 contacts, the decontamination of 25 households, the opening of 6 rings, the securing of 4 bodies, the completion of 34 of the 40 EDS (85.0%), 22 out of 25 swabs and the announcement of 22 positive results; support for Professor Muyembe's press conference held in Bunia in front of media professionals.
- **In North Kivu**, 15,020 people were reached during 4,881 home visits, 1,280 people were reached during 58 educational talks and 3,064 during 16 mass communication actions; 201 community alerts notified out of 450 expected (44.6%), 6 feedbacks resolved out of 9 (66.6%), none of the 2 recorded incidents resolved and 1,262 people engaged out of 4,750 (26.5%); media coverage of the unloading of 4 recovered patients at the Katwa CTE and mass communication in the commune of Mususa in Butembo on barrier measures and the reporting of alerts.
- **In Haut-Uélé**, 25 educational talks reached 444 people, 5 mass activities reached 746 people, 48 home visits reached 478 people, and 5 community dialogues reached 20 people, for a total of 1,688 people engaged in the health zones of Isiro, Pawa, and

Boma Mangbetu and Wamba; 11 alerts of suspected cases and deaths notified by community actors (6.9%), 22 feedbacks and 2 incidents resolved within 48 hours in Isiro; a rumor concerning an alleged vaccination of students against Ebola was denied by a visit to schools, an appeal to the Sub-Proved of Isiro, an interview on RTNC and a joint broadcast on Radio Nava.

- **In Tshopo**, 732 households were visited at home, reaching 4,320 people, and an educational talk was held with 202 displaced persons from the Kanga Kanga site; 3 interactive programs and 16 public service announcements were broadcast in 8 media outlets, training for 25 journalists in Kisangani was completed, and 600 A2-sized Ebola virus disease (EVD) posters were received; 54 community alerts were notified and all resolved, and one feedback issue was resolved within 48 hours. In Bas-Uélé, 1,628 people were sensitized in schools (623 in Poko, 600 in Buta, 243 in Bondo, and 162 in Monga), along with 218 members of the Nande cultural association in Buta and 68 community leaders in Titule, as well as 102 home visits reaching 288 people related to the Bambili Ebola case.

1.8.2. Challenges

Persistent community resistance: **In Tshopo**, there is a need for handwashing facilities in schools; a confirmed case who escaped from the treatment center remains at home; and there is a shortage of posters and banners. **In Ituri**, there is insufficient transportation to support other key areas, a broken printer, and no internet connection for the key area. In Haut-Uélé, there is a complete breakdown of visibility tools and communication equipment in all affected health zones. In Bas-Uélé, roads to some health zones and areas are impassable, there is a lack of transportation and phone credit, demotivation among field workers, and poor reporting from health zones.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 34 people among the new confirmed cases and 99 of the 161 confirmed cases under follow-up (61.5%) benefited from SMSPS interventions, as well as 30 of the 82 new suspects of the day (36.6%) and 105 of the 147 suspects under follow-up (71%); 87 results announced, including 34 positive cases, 287 caregivers and 45 PPLs supported, 38 children followed, 7/7 children followed in daycare, 9 of the 13 households of separated and orphaned children supported and 13 of the 26 recovered individuals followed in the community (50%); 6 of the 10 notified health zones are covered, for 10 of the 28 health zones.
- **In North Kivu**, 21 people among the new confirmed cases and 197 confirmed cases under follow-up were taken care of, as well as 194 new suspected cases and 228 suspected cases under follow-up; 89 results announced, including 12 positive cases, 65 PPLs supported, 66 households accompanied and 11 of the 12 recovered are followed up in the community; 27/27 children followed up in the community and 1/1 in nursery; 373 people (102 men, 173 women and 98 children) benefited from psychoeducation sessions with the support of UNICEF and the 5 notified health zones are covered.
- **In Tshopo**, 3 confirmed cases under follow-up, 2 new suspected cases and 2 suspected cases under follow-up have benefited from SMSPS interventions; one result announcement made, 9 households supported and 2 PPLs supported; 2 of the 11 recovered have been reintegrated and 2 of the 4 notified health zones are covered.

In Haut-Uélé, one new confirmed case and 12 confirmed cases under follow-up received SMSPS interventions, as well as two new suspected cases and five suspected cases under follow-up. Eleven laboratory results were announced, including one positive result. Fifty-two caregivers at the Ebola Treatment Center (ETC) and 27 people with HIV/AIDS (PHI) received support. Five separated children received community support, and nine household and community members received support. The health zones of Pawa and Boma Mangbetu did not submit their data. In Bas-Uélé, support for the confirmed case of Ganga, his family, and identified contacts remains limited by the lack of a psychologist in the health zone.

1.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (10/28 health zones covered). **In North Kivu**, low coverage of interventions around the latest confirmed cases in the health zones of Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi; insufficient human resources and motivation. **In Haut-Uélé**, difficulty in the mobility of community health workers in Wamba and Boma Mangbetu; staff demotivation due to lack of payment. **In Tshopo**, absence of SMSPS providers in Wanie-Rukula and Bafwasende and low partner participation. **In Bas-Uélé**, absence of a psychologist in the Ganga health zone.

1.10. Logistics

1.10.1. Main actions

- **In Bas-Uélé**: mobility ensured for the rapid intervention team deployed in the Ganga health zone and for the health zone teams responsible for verifying alerts and active search.
- **In North Kivu**: 11 ambulances available, of which 5 are in use (45%), with 11 calls received and 11 missions completed, all successful (100%); the Musienene ambulance received no calls in the last 24 hours. • **In Tshopo** : receipt of supplies from the VILLAGE REACH allocation and IPC kits from CARITAS, with packaging and shipment to Bafwandambo; monitoring of the construction of the Madula isolation ward and the Bafwasende Ebola Treatment Center; support for the mobility of the IPC, PEC, Communication, PoE/PoC, and vaccination teams; receipt of 600 A2 posters from Jhpiego.
- **In Haut-Uélé**: receipt of 522 kg of care and IPC supplies and equipment provided by WHO and unloading of a truckload of 7,646 kg of care kits and PPE provided by UNICEF; delivery of an IPC kit for training in Niangara and chlorine to PoCs not covered by IOM; monitoring of the construction of the Isiro CTE and fuel balance of 520.
- **In South Ubangi**: delivery of some PCI inputs and monitoring tools for the Bulu Health Zone; deployment of an EIR.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, the pillar provided support and security coverage for IPC teams for household decontamination in the health districts of Bundji and Mangote, the La Fraternité health center (Mabolio health district), the Maranatha dispensary (Kasanga health district), the Therapeutic Medical Center (Mandradele health district), the Salama dispensary and the Butanuka health center (Butanuka health district), as well as EDS teams for the burial of confirmed cases in the family cemeteries of Kalibo, Makene, Bundji and Kasinga (Katwa), with 100% coverage of the interventions requested.

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team, a lack of financial incentives for agents and non-payment of police officers and security agents assigned to guard the ESS; lack of support from security elements in several PoCs along the Butembo axis (Kangote, Kyaghala, Mususa and Komba); delayed payment of actors involved in the response and non-compliance with commitments by some partners in several health zones.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions

- **In Ituri**, awareness-raising on the PSEAH in the Bunia and A health zones targets traders, taxi drivers, students of SHALOM University, women entrepreneurs and members of CEMADEF; 287 participants including 193 women among whom 3 PLH, 39 men, 50 girls and 5 boys.
- **South Kivu**, induction of 2 staff members, 1 man and 1 woman, on EAS/HS concepts, reporting mechanisms (151 and 49 55 55) and complaints management and reporting mechanisms (ZS Uvira).

- **NORTH KIVU**, Briefing on EAS/HS Concepts, reporting mechanisms (151 and 49 55 55) and Complaint Management organized in the Goma and Mabalako Health Zones for the benefit of 3 staff, 5 investigators from the Tumaini 2 project, supported by the IMC and 53 service providers from the Mabalako Health Zone; 61 people briefed including 14 women, 47 men.
- **Haut-Uélé**: briefing on the PSEAH in the Gombari Health Zone for members and IT staff of the various AS, for 34 participants (5 women and 29 men): 34 codes of good conduct signed; awareness-raising on the PSEAH in the Boma Mangbetu and Wamba Health Zones targeting students and teachers at the Boma Institute, and the population of the Maman Kahenga AS (264 beneficiaries including 31 women, 16 men, 102 girls and 115 boys).

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- In the provinces of Ituri, North Kivu and Haut-Uélé, sustained transmission is noted while the provinces of Tshopo and Bas Uélé show sporadic notification.
- In the last 24 hours, **59 new confirmed cases**, including **16 community deaths and 16 intra-CTE deaths**, have been reported, bringing the national total to **7,404 confirmed cases and 3,577 deaths (case fatality rate: 48.3%)**.
- Ituri remains the epicenter of the epidemic, accounting for **77.7%** of confirmed cases and **59.3%** of new cases reported in the last 24 hours.
- The performance of epidemiological investigations remains satisfactory, with 100% of suspected cases investigated and a stable contact tracing rate of **87.4%**.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.
- Strengthen preparedness and response to Ebola virus disease in the provinces of the river corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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